

SHSS

Social & Human Support Services

QUALITY ASSURANCE MANUAL

Residential Care — Adult & Child and Youth Services

MCFD | Community Living BC (CLBC) | Community Care Licensing

Document Control Information

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Section 1: Purpose and Scope

1.1 Purpose

This Quality Assurance (QA) Manual establishes the framework, policies, and procedures by which Social & Human Support Services (SHSS) monitors, evaluates, and continuously improves the quality of residential care services provided to persons served. This manual applies to both Adult Residential Care (group homes and supportive living) and Child and Youth Residential Care programs operated by SHSS.

This manual is developed in alignment with the requirements of the Ministry of Children and Family Development (MCFD), Community Living British Columbia (CLBC), and the Community Care Licensing Branch of the BC Ministry of Health.

1.2 Legislative and Regulatory Framework

SHSS operates under the following legislation and regulations. All quality assurance activities are designed to demonstrate compliance with these requirements:

Legislation / Regulation	Governing Body	Relevance to SHSS	Oversight Contact
Community Care and Assisted Living Act (CCALA)	BC Ministry of Health	Licensing of residential care facilities; operator responsibilities	Community Care Licensing Branch
Child, Family and Community Service Act (CFCSA)	MCFD	Standards for child and youth residential care; rights of children in care	MCFD Director of Child Welfare
Community Living Authority Act	CLBC	Delivery of community living supports to adults with developmental disabilities	CLBC Regional Operations
Residential Care Regulation (BC Reg. 96/2009)	BC Ministry of Health	Physical environment, staffing, care standards for residential facilities	Community Care Licensing Officers
Adoption Act / Child in Home of a Relative	MCFD	Applicable to specific placement types	MCFD Regional Director
Personal Information Protection Act (PIPA)	BC Privacy Commissioner	Protection of personal information of persons served and staff	Privacy Officer — SHSS

Workers Compensation Act / OHS Regulation	WorkSafeBC	Health and safety of workers and persons served	OHS Representative — SHSS
Mental Health Act (BC)	BC Ministry of Health	Rights and care of persons detained or receiving involuntary treatment	Clinical Director — SHSS

1.3 Scope of This Manual

This manual applies to:

- All residential programs operated by SHSS, including adult group homes, supportive living residences, and child and youth care homes
- All SHSS personnel — full-time, part-time, on-call, contracted, and volunteer — working in residential settings
- All persons served in SHSS residential programs, including adults with developmental disabilities (CLBC-funded) and children and youth in the care of MCFD
- All residential sites licensed or registered under applicable BC legislation
- All contracted service providers and community partners involved in residential service delivery

1.4 Mission, Vision, and Values

MISSION	To provide safe, nurturing, and person-centred residential care that supports individuals to live with dignity, belonging, and the highest possible quality of life.
VISION	A community where every person — regardless of age, ability, or circumstance — has a place to call home and people who care.
VALUES	Safety • Dignity • Respect • Inclusion • Accountability • Continuous Improvement • Family and Community Connection

Section 2: Governance and Leadership

2.1 Leadership Accountability

The Executive Director is ultimately responsible for the quality and safety of all residential care services delivered by SHSS. Leadership responsibilities in relation to quality assurance include:

- Ensuring all residential programs operate in compliance with applicable legislation, licensing requirements, and MCFD/CLBC contractual obligations
- Allocating sufficient resources — staffing, training, and budget — to maintain a safe and high-quality residential environment
- Reviewing QA reports at minimum quarterly; presenting annual QA summary to the Board of Directors
- Ensuring prompt corrective action when licensing, compliance, or quality concerns are identified
- Maintaining open communication with MCFD Regional Directors, CLBC Regional Managers, and Community Care Licensing Officers

2.2 Quality Assurance Committee (QAC)

The Quality Assurance Committee is the primary governance body responsible for overseeing QA activities across all SHSS residential programs.

QAC Attribute	Detail
Chair	Quality Assurance Manager
Members	Executive Director, Clinical/Care Director, HR Manager, Residential Program Supervisors (each site), Person-Served/Family Representative, MCFD/CLBC Liaison (as available)
Meeting Frequency	Monthly; additional meetings as required by critical incident, licensing concern, or ministry directive
Quorum	Majority of voting members
Reporting To	Board of Directors via Executive Director
Minutes	Recorded, reviewed, and retained for minimum 7 years

2.3 Board of Directors Oversight

The Board of Directors provides strategic and fiduciary oversight, including:

- Annual approval of this QA Manual
- Quarterly review of QA summary and incident reports
- Oversight of organizational risk related to licensing, compliance, and ministry relations
- Review and approval of corrective action plans arising from licensing inspections or ministry reviews

Section 3: Licensing and Regulatory Compliance

3.1 Community Care Licensing — Adult Residential

All SHSS adult residential facilities are licensed under the Community Care and Assisted Living Act and the Residential Care Regulation. Licensing compliance requirements include:

- Valid operating licence posted and maintained at each facility at all times
- Immediate notification to the Licensing Officer of any change in operator, manager, or facility capacity
- Maintenance of physical environment, equipment, and safety systems to licensing standards
- Staff-to-resident ratios maintained at or above the minimum required by the Residential Care Regulation
- All required health and safety inspections completed and records maintained on site
- Cooperation with all Community Care Licensing Officer inspections — scheduled and unannounced
- Written response to any licensing inspection report within the required timeframe

3.2 MCFD — Child and Youth Residential Compliance

SHSS child and youth residential programs operate under contracts with MCFD and must comply with the Child, Family and Community Service Act (CFCSA) and all applicable MCFD standards and directives. Key requirements include:

- Maintaining current contracts and Service Level Agreements with MCFD
- Adhering to MCFD Standards for Child and Youth in Care — Residential Services
- Ensuring all children and youth have an active Individual Service Plan (ISP) / Support Plan reviewed at MCFD-specified intervals
- Compliance with MCFD reporting requirements for critical incidents, abuse allegations, and missing youth
- Participation in MCFD file reviews and program audits
- Maintaining a current and approved Emergency Response Plan for each home
- Supporting children and youth to exercise their rights under the CFCSA

3.3 Community Living BC (CLBC) — Adult Disability Services

SHSS adult services funded through CLBC must comply with CLBC's Standards and Guidelines for Community Living Services, including:

- Maintaining current Individual Service Plans (ISPs) for all CLBC-funded persons served
- Reporting critical incidents to CLBC within required timelines
- Participation in CLBC Quality Service Reviews (QSRs) and site monitoring
- Demonstrating person-directed planning and self-determination in all service decisions
- Compliance with CLBC's Abuse Prevention and Response Protocol
- Annual submission of required outcome data and service reports to CLBC

3.4 Compliance Monitoring Schedule

Compliance Activity	Frequency	Responsible	Evidence
Licensing Inspection Readiness	Ongoing	QA Manager / Site Supervisors	Internal inspection checklist
Residential Care Regulation Review	Annual (January)	QA Manager	Compliance review log
MCFD Contract Compliance Review	Per contract schedule	Program Director	Contract reporting submissions
CLBC Service Review Preparation	Prior to each QSR	QA Manager	QSR preparation binder
Incident Reporting Compliance Audit	Monthly	QA Manager	Incident log analysis
ISP / Support Plan Currency Check	Monthly	Program Supervisors	Plan review tracking sheet
Privacy Policy Review	Annual	Privacy Officer	Policy review log
Emergency Plan Review & Drill	Annual (drill semi-annual)	Site Supervisors	Drill records
Medication Administration Audit	Monthly	Nurse/Designate	MAR audit tool
Personnel File Compliance Check	Quarterly	HR Manager	Personnel file checklist

Section 4: Person-Centred Care and Rights of Persons Served

4.1 Rights of Adults in Residential Care

SHSS upholds the rights of all adults in residential care as established under the Residential Care Regulation and CLBC's Person-Centred Standards. These rights include:

- The right to be treated with respect and dignity at all times
- The right to privacy — personal space, correspondence, and personal health information
- The right to make decisions about their own lives and services, including the right to refuse services
- The right to participate in developing and reviewing their own Individual Service Plan
- The right to have family members and advocates involved in planning (with consent)
- The right to receive visitors and maintain community connections
- The right to raise concerns and access the complaint process without fear of consequences
- The right to manage their own finances, with supports as needed

4.2 Rights of Children and Youth in Care (CFCSA)

All children and youth in care of MCFD placed with SHSS have the following rights under the Child, Family and Community Service Act:

- The right to be informed about decisions that affect them, in language they can understand
- The right to participate in decisions about their care, including placement and service planning
- The right to cultural identity — to know and be connected with their heritage, especially for Indigenous children and youth
- The right to access a Representative for Children and Youth (RCY) advocate
- The right to be free from physical, emotional, and sexual abuse
- The right to education, health care, and support for their individual needs
- The right to have a trusted adult (caregiver, family member, or advocate) support them

INDIGENOUS RIGHTS

SHSS is committed to full implementation of the Declaration on the Rights of Indigenous Peoples Act (DRIPA) and Jordan's Principle. For Indigenous children, youth, and adults in our care, SHSS ensures culturally safe services, connection to Indigenous community and culture, and meaningful involvement of Indigenous communities in service planning.

4.3 Individual Service Planning

Every person served in an SHSS residential program has an active, current Individual Service Plan (ISP) or Support Plan. ISP standards include:

- Developed collaboratively with the person served, their family/guardian (where appropriate), and relevant service providers
- Reflects the person's goals, strengths, preferences, and areas requiring support
- Reviewed and updated at MCFD- or CLBC-specified intervals (minimum annually; more frequently if needs change)

- Includes specific, measurable goals with identified supports and timelines
- Consistent with trauma-informed, culturally safe practice
- Copy provided to the person served and their representative

Section 5: Safe Home Environment

5.1 Physical Environment Standards

All SHSS residential facilities are maintained to provide a safe, clean, comfortable, and home-like environment consistent with licensing standards and MCFD/CLBC requirements. Physical environment requirements include:

- Bedrooms meet minimum size and furnishing requirements per the Residential Care Regulation
- All bathrooms, common areas, and outdoor spaces are clean, safe, and accessible
- All appliances, fixtures, and equipment maintained in good working order
- Hazardous materials (chemicals, medications) stored securely and out of reach of persons served
- Emergency evacuation routes posted visibly at each floor
- Smoke detectors, carbon monoxide detectors, and fire extinguishers inspected monthly
- All modifications for accessibility in place and functional

5.2 Monthly Safety Inspection

Site Supervisors conduct a documented monthly safety inspection at each residential facility. The inspection covers:

- Fire safety equipment (smoke detectors, extinguishers, sprinklers if applicable)
- Medication storage and security
- General cleanliness and maintenance of all living areas
- Outdoor areas — fencing, play equipment (child/youth homes), lighting
- Kitchen safety — appliances, food storage temperatures, sanitation
- Emergency contact lists and emergency plans — current and posted
- Vehicle safety check (where applicable)

Completed inspection checklists are reviewed by the QA Manager monthly and filed for licensing review.

5.3 Emergency Preparedness

Each SHSS residential home maintains a current Emergency Response Plan (ERP), reviewed annually and tested through:

- Fire drills — minimum twice per year at each site (different times of day)
- Earthquake / emergency evacuation drill — annually
- Tabletop exercise with staff — annually

Drill records, including date, time, number of participants, and any issues identified, are maintained on site and submitted to the QA Manager.

Section 6: Staffing and Human Resources

6.1 Hiring Requirements — Residential Care

All personnel working in SHSS residential programs must meet the following minimum requirements prior to commencing work:

Requirement	Details
Criminal Record Check	Clear Criminal Record Check with Vulnerable Sector Screening (VSS) — mandatory for all staff with access to persons served; renewed every 5 years or as required by MCFD/licensing
Child Welfare Record Check	Required for all staff in MCFD-contracted child and youth homes
First Aid / CPR	Standard First Aid with CPR-C (Level C), or as specified by licensing — maintained current at all times
Education / Credentials	Minimum as required per program type: Child and Youth Care Worker, Community Support Worker, or equivalent recognized credential, or combination of education and relevant experience per MCFD/CLBC requirements
Reference Checks	Minimum two professional references verified and documented prior to hire
Confidentiality Agreement	Signed prior to first day of work
Driver's Abstract	Required for all staff who transport persons served; current abstract on file; renewed annually

6.2 Orientation and Probationary Period

All new residential care staff complete a structured orientation within their first 30 days of employment, covering:

- SHSS mission, values, and organizational overview
- Overview of governing legislation and regulatory requirements (MCFD, CLBC, licensing)
- Rights of persons served — adults and children/youth
- Mandatory abuse reporting obligations under the CFCSA and the Adult Guardianship Act
- Individual Service Plan review for each person in the home
- Medication administration procedures (if applicable to role)
- Emergency procedures and evacuation routes specific to their site
- Incident reporting — process and timelines
- Behaviour support planning and de-escalation
- Trauma-informed and culturally safe practice
- Documentation standards and home log requirements

- Completion of all mandatory training listed in Section 6.3

6.3 Mandatory Training — Residential Care Staff

The following training is mandatory for all residential care personnel. HR tracks completion and reports to the QA Manager monthly. Completion is a condition of ongoing employment.

Training	Initial Deadline	Renewal	Evidence
Trauma-Informed Practice	Within 30 days of hire	Annual	Certificate / LMS record
Indigenous Cultural Safety & Humility	Within 30 days of hire	Annual	Certificate (e.g., San'yas)
Mandatory Reporting — Child Abuse & Neglect (CFCSA)	On hire	Annual	Signed acknowledgement / certificate
Mandatory Reporting — Adults (Adult Guardianship Act)	On hire	Annual	Signed acknowledgement / certificate
WHMIS / Safe Chemical Handling	On hire	Annual	Training record
Medication Administration (applicable roles)	Before administering medications	Annual + on medication change	Competency sign-off
Non-Violent Crisis Intervention (NVCi) / Safety Care	Within 60 days of hire	Biennial (or as expiry requires)	Certification
Standard First Aid + CPR-C	On hire	Biennial	Current certificate on file
Emergency Procedures (site-specific)	During orientation	Annual drill participation	Drill attendance record
Abuse Prevention and Response Protocol (CLBC)	Within 30 days of hire	Annual	Signed attestation
Privacy and	On hire	Annual	Signed acknowledgement

Confidentiality (PIPA)			
Behaviour Support Planning Fundamentals	Within 60 days of hire	Annual	Training completion record
Diversity, Equity, and Inclusion	On hire	Annual	LMS certificate
Fire Safety Training (site-specific)	During orientation	Annual	Training record

6.4 Staffing Ratios

Staffing ratios are maintained in accordance with licensing requirements, MCFD/CLBC contract requirements, and the assessed needs of persons served. Ratio requirements include:

- Minimum staffing ratios per the Residential Care Regulation maintained at all times
- Staffing plans reviewed and adjusted when a new person is admitted with complex needs
- On-call and backup staff available at all times to ensure ratio compliance
- Staffing ratio variances documented and reported to the QA Manager immediately

6.5 Performance Management

All residential care personnel participate in the following performance management process:

- 90-day check-in — informal review with immediate supervisor
- Annual performance appraisal — completed by supervisor, reviewed with employee, signed, and placed on personnel file
- Individual development plan — created collaboratively and reviewed annually
- Progressive discipline — applied consistently per HR Policy, with documentation at each stage
- Recognition and appreciation — peer and supervisor nominations reviewed at team meetings

Section 7: Medication Management

7.1 Medication Administration Standards

Medications for persons served are managed in accordance with the Residential Care Regulation, MCFD standards, CLBC guidelines, and physician/nurse practitioner orders. Key standards include:

- All medications stored in a locked, secure location accessible only to authorized staff
- Prescription medications dispensed only as per the written order of a licensed prescriber
- Over-the-counter medications administered only with documented authorization (physician order or consent)
- Medication Administration Records (MARs) completed accurately at the time of administration
- No medication administered without a current, signed MAR on file
- Medications are never crushed, altered, or administered in food without explicit written prescriber authorization

7.2 Medication Administration Competency

Only staff who have successfully completed medication administration training and competency assessment may administer medications. Competency is assessed:

- Prior to administering medications independently (initial sign-off by Nurse or Designate)
- Annually thereafter, or following any medication error, or change in a person's medication regime

7.3 Medication Errors

Any medication error — including wrong medication, wrong dose, wrong time, wrong person, or missed dose — is treated as a reportable event:

1. Staff member immediately notifies the Site Supervisor and documents the error
2. Poison Control and/or prescribing physician notified as appropriate
3. Incident Report completed within 24 hours
4. Person served and their representative/guardian notified as appropriate
5. Ministry/CLBC notification as required by reporting protocol
6. Root cause analysis conducted for recurring or serious errors
7. Corrective action implemented and monitored by QA Manager

7.4 Monthly Medication Audit

The QA Manager or designated Nurse conducts a monthly medication audit at each residential site, reviewing: MAR accuracy and completeness, medication storage conditions, expiry dates, controlled substance counts, and disposal records. Audit results are reviewed by the QAC monthly.

Section 8: Critical Incident Reporting and Review

8.1 Definition of Critical Incidents

A critical incident is any unexpected or unplanned event that has resulted in, or could reasonably have resulted in, harm to a person served, staff member, or member of the public. Critical incidents include but are not limited to:

Incident Type	Description
Serious injury	Any injury requiring medical treatment beyond basic first aid
Alleged abuse or neglect	Any allegation of physical, sexual, emotional, or financial abuse or neglect by staff, another person served, or a community member
Missing person	Any person served who cannot be accounted for and is not at an expected location
Attempted suicide / self-harm	Any act or expression of suicidal intent or serious self-harm
Death	Death of any person served, in any circumstance
Medication error	Any error in medication administration as described in Section 7.3
Restraint use	Any use of physical intervention or restraint — including emergency interventions
Fire / emergency evacuation	Any fire, gas leak, or emergency requiring evacuation
Criminal activity	Any incident involving criminal activity at a residential site
Elopement	Any instance of an individual leaving the residence without authorization or supervision

8.2 Reporting Timelines

SHSS meets or exceeds all mandatory reporting timelines required by MCFD, CLBC, and Community Care Licensing:

Report Type	Timeline	Responsible Party
Immediate verbal report	Immediately upon discovery	Site Supervisor → Program Director → QA Manager
Internal Incident Report form	Within 24 hours	Staff member who witnessed / discovered incident

MCFD Critical Incident Report	Within 24 hours (child/youth)	Program Director / QA Manager
CLBC Critical Incident Report	Within 24 hours (adults)	Program Director / QA Manager
Community Care Licensing Notification	As required per incident type	Executive Director / QA Manager
Police notification	Immediately for crimes, missing persons, or child abuse allegations	Site Supervisor with Program Director authorization
Coroner notification	Immediately in event of death	Executive Director
Family/Guardian notification	Within 24 hours (or sooner as appropriate)	Program Supervisor / Director

8.3 Incident Review Process

Every critical incident is reviewed through the following process:

8. Initial review by Site Supervisor and Program Director within 2 business days
9. QAC review within 5 business days of incident report receipt
10. Root Cause Analysis (RCA) completed for all serious incidents — injury, death, abuse allegation, restraint use
11. Corrective Action Plan (CAP) developed and assigned within 10 business days
12. CAP implementation monitored monthly by QA Manager
13. CAP closed when corrective action is verified as effective
14. Monthly trending analysis — incident type, time, location, staff involved — reported to QAC
15. Annual incident summary presented to Board of Directors

Section 9: Abuse Prevention and Response

9.1 Commitment to Zero Tolerance

SHSS has a zero-tolerance policy for abuse and neglect of any person served. This applies to all forms of abuse — physical, sexual, emotional, financial, and neglect — by any staff member, volunteer, contractor, or peer. Any substantiated act of abuse will result in immediate termination of employment or contract, and referral to appropriate authorities.

9.2 Mandatory Reporting Obligations

All SHSS personnel — including volunteers and contractors — are mandatory reporters under:

- Child, Family and Community Service Act (CFCSA) — Section 14: Duty to report child abuse, neglect, or risk of harm. Reporting is immediate to MCFD and/or police. Failure to report is a criminal offence.
- Adult Guardianship Act — Duty to report abuse or neglect of adults who are incapable of protecting themselves.

Mandatory reporting obligations are explained during orientation and reviewed annually. No supervisor or manager has the authority to instruct a staff member not to report.

9.3 Abuse Prevention Measures

SHSS implements the following systemic abuse prevention measures:

- Criminal record and child welfare checks completed for all staff prior to hire
- Behaviour Support Plans in place for all persons with identified behaviours of concern — ensuring staff have safe, non-restrictive strategies
- Two-staff presence policy for high-risk care activities (bathing, personal care) where feasible
- Regular review of home environment for isolation risks
- Persons served informed of their rights and how to access advocacy support
- Anonymous reporting mechanism available to all staff and persons served
- Quarterly self-assessment of abuse prevention practices by each residential site

9.4 Responding to Allegations

When an allegation of abuse or neglect is received:

16. Staff member receiving the allegation reports immediately to their supervisor
17. Supervisor ensures the safety of the person served and removes the alleged perpetrator from contact pending investigation
18. Program Director notified immediately; decision made regarding whether to contact police
19. MCFD / CLBC / Licensing notified as required by reporting timelines
20. Executive Director notified within 2 hours
21. Internal investigation initiated — conducted by a person with no conflict of interest
22. Accused staff member placed on administrative leave (with or without pay per HR policy) pending investigation

- 23. Outcome of investigation documented; disciplinary action as warranted
- 24. Lessons learned reviewed by QAC; system changes implemented as needed

Section 10: Behaviour Support Planning

10.1 Positive Behaviour Support Approach

SHSS is committed to a Positive Behaviour Support (PBS) approach in all residential programs. PBS is grounded in understanding the function and meaning behind behaviour, and ensuring the environment and interactions meet the person's needs proactively. The use of restrictive or aversive interventions is not permitted except in emergency situations as outlined below.

10.2 Behaviour Support Plans (BSPs)

All persons served with identified behaviours that may pose risk to themselves or others have a current, written Behaviour Support Plan developed by a qualified Behaviour Interventionist or Clinical Supervisor in consultation with the person served, their family/guardian, and residential staff. The BSP includes:

- Description of the behaviour — frequency, intensity, triggers
- Hypothesized function of the behaviour (what need it serves)
- Proactive strategies — environmental modifications, teaching replacement behaviours
- De-escalation strategies — specific to the person
- Crisis response plan — including when and how to contact emergency services
- Data collection requirements — how staff track behaviour occurrence
- Review schedule — minimum quarterly, or following any significant behavioural incident

10.3 Restrictive Interventions

Physical intervention and restraint are used only as a last resort in emergency situations where there is an imminent risk of serious harm. SHSS policy requires:

- Only staff trained and currently certified in an approved physical intervention program (e.g., Safety-Care, NVCi) may use physical intervention
- Every use of physical intervention is documented as a critical incident (Section 8)
- Debrief conducted with all involved staff and (where possible) the person served within 24 hours
- Review by QAC within 5 business days
- Pattern of restraint use triggers immediate review of BSP and environmental factors

Section 11: Health Care Management

11.1 Health Care Planning

Every person served has a Health Care Plan as part of their Individual Service Plan. The Health Care Plan documents:

- Current diagnoses and medical history relevant to residential care
- Scheduled medical appointments and follow-up requirements
- Allergies — food, medication, environmental
- Current medications and administration schedule
- Specific health monitoring requirements (e.g., blood pressure, blood glucose, seizure monitoring)
- Emergency health protocols — including what to do in a medical emergency
- Health care provider contacts

11.2 Medical Appointments and Health Monitoring

Residential care staff are responsible for:

- Ensuring persons served attend scheduled medical, dental, and specialist appointments
- Accompanying persons served to appointments as needed and documenting outcomes in the home log and Health Care Plan
- Monitoring and documenting health indicators as outlined in each person's Health Care Plan
- Communicating health concerns to the Site Supervisor and, where necessary, to the person's family/guardian and health care provider

11.3 Infection Prevention and Control

SHSS residential homes maintain infection prevention and control (IPAC) protocols including:

- Routine hand hygiene practices for all staff and persons served
- Isolation protocol for communicable illness
- Cleaning and disinfection schedule for all common areas, bathrooms, and high-touch surfaces
- Personal protective equipment (PPE) available and used as required
- Immunization records maintained for all persons served (where applicable and consented)

Section 12: Stakeholder Input and Satisfaction

12.1 Feedback Mechanisms

SHSS actively solicits and responds to feedback from persons served, families, guardians, MCFD/CLBC representatives, and community partners through the following mechanisms:

Mechanism	Description
Satisfaction Surveys	Conducted at minimum annually with persons served and their families/guardians; available in accessible formats
Resident Meetings	Monthly home meetings at each residential site where persons served can raise concerns, make suggestions, and participate in home decisions
Family Advisory Input	Annual meeting with family members/guardians to discuss program quality and areas for improvement
Advocacy Access	All persons served have information about how to access an independent advocate; MCFD Representative for Children and Youth (RCY) information posted in youth homes
Suggestion Boxes	Available at each residential site and the main office
Exit Interviews / Transition Surveys	Completed when a person transitions out of SHSS residential care
Ministry / CLBC Feedback	Licensing inspection reports, MCFD program reviews, and CLBC Quality Service Review findings reviewed by QAC within 30 days of receipt

12.2 Satisfaction Targets

- Overall satisfaction with residential services: Target $\geq 85\%$
- Satisfaction with respect and dignity shown by staff: Target $\geq 90\%$
- Satisfaction with safety of the home environment: Target $\geq 90\%$
- Persons served report feeling heard and involved in decisions: Target $\geq 80\%$
- Families/guardians satisfied with communication from SHSS: Target $\geq 85\%$

12.3 Responding to Feedback

All feedback — positive and negative — is logged, reviewed by the QAC monthly, and used to inform quality improvement. Where feedback indicates a concern, the QA Manager initiates a formal review within 10 business days. Persons served and families are informed of actions taken in response to their input.

Section 13: Grievance and Complaint Process

13.1 Right to Complain

All persons served, their families, guardians, and advocates have the right to raise concerns and make complaints without fear of consequences to their care or placement. This right is communicated at admission, posted visibly in every home, and reinforced by all staff.

13.2 Internal Complaint Procedure

25. Person served, family, or guardian raises concern verbally or in writing to any staff member
26. Staff records the concern immediately and informs the Site Supervisor within 24 hours
27. Site Supervisor attempts informal resolution and acknowledges the concern to the complainant within 2 business days
28. If unresolved within 5 business days, Site Supervisor escalates to Program Director
29. Program Director conducts a formal review and provides a written response within 10 business days
30. If still unresolved, the person may request review by the Executive Director — response within 10 business days
31. At any stage, the person is informed of their right to contact MCFD, CLBC, Community Care Licensing, or the Representative for Children and Youth (for youth in care)

13.3 External Complaint Contacts

Agency / Issue	Contact Information
Community Care Licensing (adults / facility concerns)	BC Ministry of Health — 1-877-952-1555
MCFD (child and youth in care)	MCFD Regional Director — local MCFD office
CLBC (adults with developmental disabilities)	CLBC Regional Manager — 1-877-660-2522
Representative for Children and Youth	1-800-476-3933 (BC toll-free)
BC Office of the Human Rights Commissioner	604-775-2000
BC Ombudsperson	1-800-567-3247
Privacy Complaints (PIPA)	Office of the Information and Privacy Commissioner — 250-387-5629

Section 14: Records Management and Documentation

14.1 Documentation Standards

All records pertaining to persons served and residential operations are maintained accurately, completely, and in accordance with MCFD, CLBC, and licensing requirements. Documentation standards include:

- All entries in the home log, MAR, and person-served records are completed at the time of occurrence — never reconstructed after the fact
- Entries are legible, dated, timed, and signed by the recording staff member
- Corrections: single strikethrough, dated, and initialled — no whiteout or deletions
- Electronic records maintained in SHSS-approved systems with appropriate access controls
- Records available for inspection by MCFD, CLBC, and Licensing Officers on request

14.2 Retention Requirements

Record Type	Retention Period
Person-served records (adults)	Minimum 7 years from date of last service
Person-served records (children/youth)	Minimum 10 years after the person turns 19 (age of majority in BC)
Critical incident reports	Minimum 7 years
Medication administration records	Minimum 5 years from date of administration
Personnel files	Minimum 7 years after end of employment
QA committee minutes	Minimum 7 years
Licensing inspection reports	Maintained for the life of the licence + 7 years

14.3 Privacy and Confidentiality

All records pertaining to persons served are protected under the Personal Information Protection Act (PIPA) of British Columbia and MCFD/CLBC privacy requirements. SHSS ensures:

- Informed written consent obtained prior to any disclosure of personal information to third parties
- Records stored securely — locked filing cabinets (paper), encrypted and password-protected systems (electronic)
- Access to records limited to personnel with a need-to-know
- Privacy breach protocol activated immediately upon discovery of any unauthorized disclosure
- Annual privacy training for all staff

Section 15: Performance Improvement

15.1 Performance Improvement Philosophy

Performance improvement at SHSS is an ongoing, data-driven process that engages all staff in continuously raising the quality and safety of residential care. We learn from incidents, inspections, feedback, and data — and we act on what we learn.

15.2 Key Performance Indicators — Residential Care

KPI	Target	Frequency	Data Source
Critical incident rate (per 1,000 resident days)	Trending downward year-over-year	Monthly	Incident log
Medication error rate	< 1 per 1,000 administrations	Monthly	MAR audit
Mandatory training completion rate	100% for all required training	Monthly	HR training log
Criminal record check currency	100% current at all times	Monthly	HR personnel files
ISP / Support Plan currency	100% current at all times	Monthly	File audit
Licensing inspection outcomes	Zero unresolved compliance orders	Per inspection	Licensing reports
Satisfaction — persons served	≥85% satisfied or very satisfied	Annual	Satisfaction survey
Satisfaction — families/guardians	≥85% satisfied or very satisfied	Annual	Satisfaction survey
Staff turnover rate	≤25% annually	Quarterly	HR records
Restraint use rate	Trending downward	Monthly	Incident / BSP data
Emergency drill completion	100% of required drills completed on schedule	Semi-annual	Drill records
Complaint resolution within timeline	≥95% resolved within policy timelines	Monthly	Grievance register

15.3 Performance Improvement Plan (PIP) Process

A formal Performance Improvement Plan is initiated when a KPI falls below target for two consecutive periods, or when a significant concern is identified through an inspection, incident review, or complaint:

32. QAC identifies the performance gap with supporting data
33. Root Cause Analysis completed using structured methodology
34. Action Plan developed — specific actions, responsible parties, and target dates
35. Actions implemented and tracked in the QAC Action Log
36. Progress reviewed at each monthly QAC meeting
37. PIP closed when sustained improvement is confirmed
38. Full PIP documentation retained for minimum 7 years

15.4 Quality Improvement Activities Calendar

Month	Quality Improvement Focus
January	Legislative and regulatory review — update policies as required
February	Annual satisfaction survey launch (persons served and families)
March	Satisfaction survey analysis; QAC annual report to Board
April	Records audit — all sites
May	Staff engagement survey
June	Emergency preparedness tabletop exercise
July	Annual review of QA Manual; HR training compliance review
August	Medication audit — full cycle review across all sites
September	Criminal record check currency audit
October	ISP / Support Plan currency audit; MCFD / CLBC relationship meeting
November	Incident trend analysis — annual report preparation
December	Board of Directors QA annual summary presentation; PIP review

Section 16: Document Control and Review

16.1 Version Control

This manual is version-controlled. All revisions are dated, summarized, and approved before release. Previous versions are archived. Revisions are initiated when:

- Legislation, regulation, or MCFD/CLBC policy changes
- A licensing inspection, audit, or ministry review identifies a gap
- A quality improvement activity identifies a need for policy update
- The Board of Directors or QAC directs a review

16.2 Version History

Version	Date	Author	Summary
1.0	July 1, 2025	QA Manager	Initial issue — aligned to MCFD, CLBC, and Community Care Licensing requirements

16.3 Annual Review

This manual is reviewed annually by the QA Manager in consultation with the QAC, Program Directors, and Executive Director. Review is completed by June 30 each year. The revised manual is approved by the Board of Directors before the new effective date of July 1.

16.4 Approval and Authorization

Role	Signature / Date
Prepared By	Quality Assurance Manager _____ Date: _____
Reviewed By	Executive Director _____ Date: _____
Approved By	Board of Directors (Chair) _____ Date: _____

Appendix A: Key Regulatory and Ministry Contacts

The following contacts are maintained current by the QA Manager and updated at minimum annually.

Agency / Role	Contact Information
MCFD Regional Director (Child & Youth)	[Insert regional office name, address, phone, and contact name]
MCFD Integrated Services Manager	[Insert name and direct contact]
Community Care Licensing Officer	[Insert assigned Licensing Officer name and contact]
CLBC Regional Manager	[Insert CLBC region, name, and contact]
CLBC Facilitator / Coordinator (key contacts)	[Insert names and contacts for key CLBC facilitators]
WorkSafeBC (OHS inquiries)	1-888-967-5377 www.worksafebc.com
BC Emergency Health Services (BCEHS)	911 (emergencies)
Poison Control Centre (BC)	1-800-567-8911
Representative for Children and Youth (BC)	1-800-476-3933
Community Care Licensing Branch (general)	1-877-952-1555
BC Coroners Service	1-877-660-3955

Appendix B: Glossary of Key Terms

Term	Definition
CCALA	Community Care and Assisted Living Act — BC legislation governing the licensing of residential care facilities
CFCSA	Child, Family and Community Service Act — BC legislation governing child protection, children in care, and residential standards for youth
CLBC	Community Living British Columbia — provincial Crown agency that funds and plans supports for adults with developmental disabilities
Behaviour Support Plan (BSP)	A written plan developed by a qualified practitioner that identifies the function of challenging behaviour and provides proactive and reactive strategies for staff
Critical Incident	Any unexpected or unplanned event that has resulted in, or could reasonably have resulted in, harm to a person served, staff, or community member
Individual Service Plan (ISP)	A written plan developed collaboratively with a person served that documents their goals, strengths, support needs, and how services will be delivered
IPAC	Infection Prevention and Control — practices and protocols to prevent the spread of communicable illness in residential settings
MAR	Medication Administration Record — the document used to record all medication administrations for a person served
MCFD	Ministry of Children and Family Development — BC ministry responsible for child protection, children in care, and contracted residential services for children and youth
Mandatory Reporter	Any person required by law to report suspected child abuse or neglect (CFCSA) or abuse/neglect of a vulnerable adult (Adult Guardianship Act)
PBS	Positive Behaviour Support — an evidence-based approach to understanding and supporting individuals with challenging behaviour through proactive, person-centred strategies
Person Served	An individual receiving residential care services from SHSS
QAC	Quality Assurance Committee — the governance body responsible for overseeing quality improvement at SHSS
QSR	Quality Service Review — CLBC's process for reviewing the quality of services provided to individuals funded by CLBC

Residential Care Regulation	BC Reg. 96/2009 — the regulation under CCALA setting out standards for licensed residential care facilities
SHSS	Social & Human Support Services — the organization to which this manual applies

End of Quality Assurance Manual — Version 1.0